

**STATE OF VERMONT
SECRETARY OF STATE
OFFICE OF PROFESSIONAL REGULATION
BOARD OF ALLIED MENTAL HEALTH PRACTITIONERS**

IN RE:	)	Docket No. 2023-73, 2023-139,
Gretchen E. Lewis	)	2022-244, 2022-245, 2022-246
License No. 068.0000569	)	2022-247, 2022-248

STIPULATION AND CONSENT ORDER

STIPULATION

NOW COME the State of Vermont, by and through State Prosecuting Attorney Jennifer B. Colin, and Respondent Gretchen E. Lewis, by and through her counsel Jennifer McDonald, who hereby stipulate and agree as follows:

1. This Stipulation is neither an admission of liability by Respondent nor a concession by the State of Vermont that its charges are not well-founded. To avoid delay, uncertainty, inconvenience, and the expense of protracted litigation, the Parties enter this full and final Stipulation pursuant to these Understandings and the Order below. Respondent does not dispute that the State may be able to prove these charges by a preponderance of the evidence if this matter proceeds to a merits hearing.

Board Authority

2. The Vermont State Board of Allied Mental Health Practitioners (the "Board") has authority to issue warnings or reprimands; suspend, revoke, limit, or condition current licenses; or prevent the renewal of lapsed licenses if, after disciplinary hearing, the Board finds that the Respondent has engaged in unprofessional conduct. 3 V.S.A. §129(a); 3 V.S.A. §129a; 3 V.S.A. §814(d); 26 V.S.A. Chapters 65 and 78; the Administrative Rules of the Board of Allied Mental Health Practitioners; and the Rules of the Office of Professional Regulation ("OPR").

Alleged Facts

3. Gretchen E. Lewis ("Respondent") of Derby Line, Vermont is licensed by the State of Vermont as a Clinical Mental Health Counselor under license number 068.0000569. The license was first issued on November 15, 2001 and expires on January 31, 2025.
4. Respondent is also licensed by the State of Vermont as an Alcohol and Drug Abuse Counselor ("LADAC") under license number 151.0127939. This license was first issued on January 31, 2017 and expires on January 31, 2025.
5. Respondent is a solo practitioner providing mental health services and substance abuse counseling in Newport, Vermont.

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Prior Discipline in 2021

6. In August 2021, Respondent was disciplined by the Board for engaging in unprofessional conduct that violated numerous essential standards of acceptable and prevailing practice and for failure to exercise independent professional judgment in the performance of licensed activities. *See* Stipulation and Consent Order in OPR Docket Nos. 2020-39, 2020-40, 2020-41, and 2021-38 ("2021 Order").
7. The unprofessional conduct identified in the 2021 Order arose from various circumstances involving numerous patients, including the following:
 - a. Engaging in a dual relationship in which Respondent provided and billed for counseling services to a relative;
 - b. Diagnosing an individual, who was not a client, whom Respondent had never met, and from whom Respondent had no consent, in order to assist another individual who was Respondent's client with obtaining State benefits;
 - c. Breaching confidentiality by potentially revealing that an individual was Respondent's patient while speaking about the individual in session with another client;
 - d. Engaging in a decades-long dual relationship and breaching boundaries with a disabled client that Respondent: saw twice a week for mental health counseling services; ran errands with and drove around the community on both counseling and non-counseling days; sometimes took for coffee with at a local coffee shop on non-counseling days; accompanied on visits with the client's dying father; and for whom Respondent accepted appointment as the "Alternate Agent" on the client's Durable Power of Attorney, which was in effect for over seven years.
8. The 2021 Order contained the following stipulated "Understanding:"

Respondent admits that the facts above are true and that the conditions below are necessary to protect the public.
9. In the 2021 Order, the Board reprimanded and conditioned Respondent's LCMHC license for a minimum of two years of full-time practice.
10. The 2021 Order required Respondent to complete 12 hours of pre-approved online coursework, provide notification of discipline to and employer reports from any mental health field employer, and be supervised by a pre-approved mental health practitioner with more than ten years' experience, including weekly meetings and monthly reports from the supervisor to OPR. Because Respondent is a solo practitioner, she engaged a supervisor that she meets with weekly who submits monthly reports to OPR.
11. Since the August 2021 Order was entered, OPR has received multiple complaints regarding Respondent's practice.

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Continuing Boundary and Ethical Issues with Client 1

12. In November 2021, OPR received a complaint against Respondent for circumstances arising from a situation involving Respondent and an adult female client who has medical, cognitive, and mental health issues ("Client 1").
13. Client 1 is the same client of Respondent's referred to in Paragraph 6(d) above and in the 2021 Order on pages 3 through 5. Respondent has already been disciplined by the Board for numerous ethical violations with Client 1.
14. Respondent and Client 1 entered a therapist-client relationship when Respondent worked at a Facility in the early 2000's. The counseling relationship continued when Respondent went into private practice in 2005.
15. Client 1 does not drive, uses a walker and has difficulty walking more than a short distance without assistance.
16. On October 29, 2021, Respondent drove Client 1 to a local restaurant and dropped Client 1 off there without any means of transportation to return home.
17. Client 1 remained at the restaurant for approximately two hours. Emergency Medical Services were dispatched to the restaurant where Client 1 was outside with her walker in a short-sleeved shirt and sweatpants despite temperatures in the 40's. Client 1 did not have a ride, a phone, or a coat and no taxi service was available. Client 1's home is over a mile from the restaurant.
18. Emergency Medical Services contacted Newport Police Department, as Client 1 had no way to get home and refused medical care. EMS and NPD both attempted, without success, to contact Respondent on October 29, 2021.
19. On Wednesday, February 9, 2022, OPR Investigator Kris Bowdish, along with NPD Officer Tanner Jacobs, visited Client 1 at her home to interview her regarding the incident.
20. In the interview, Client 1 reported that she had been kicked out of the restaurant referenced in Paragraph 16 above because of the incident on October 29, 2021.
21. In the interview, Client 1 reported that she and Respondent go out in Respondent's vehicle, they go for coffee, to fast food restaurants and to Client 1's brother's home. Client 1 reported that Respondent takes her out when Client 1 cannot get a taxi. Client 1 reported she sees Respondent on Mondays, Wednesdays, Fridays and Saturdays.
22. In the interview, Client 1 also informed Investigator Bowdish and Officer Jacobs that Respondent would be picking her up at noon that same day, which was a Wednesday. Client 1 was concerned that Officer Jacobs was going to arrest Respondent.

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23. Soon after Investigator Bowdish and Officer Jacobs left Client 1's residence on February 9, 2022, Client 1 called the Newport Police Department twice to speak with Officer Jacobs. Respondent was with Client 1 when she made the calls, which were recorded by the police department.
24. In the first call, Client 1 asked to speak with Officer Jacobs. While speaking to the dispatcher, Client 1 said she wanted to tell Officer Jacobs that her schedule with Respondent is Mondays, Wednesdays, Fridays, and Saturdays. At the same time, Respondent can be heard in the background saying: "It is not."
25. During the call, Client 1 also informed the dispatcher that "Gretchen is here waiting for me to go on an outing."
26. During the call, when the dispatcher put Client 1 on hold to get Officer Jacobs, Respondent and Client 1 had exchanges that included Respondent stating the following:

"We do not have Wednesdays on our schedule."
"You cannot go out on Wednesdays."
"It's not on the schedule."
"I did not tell him Wednesdays was on the schedule 'cause it's not."
"They don't want you going out with me unless it's on the schedule. It has to be on the schedule."
"You don't have a scheduled appointment on Wednesdays. We are Mondays and Fridays, that's it. Hang up the phone and please don't start any more problems." "I just can't pick you up if it's not on the treatment plan."
"I will take your garbage out for you and that's all I can do."
"It's not his [Officer Jacobs] fault. It's the investigator. That's the woman that is in charge of this. That's the one that has the power, not him. You are talking to the wrong person."
27. Client 1 disconnected the call while on hold, but then called Newport Police Department again shortly thereafter to speak with Officer Jacobs.
28. When Officer Jacobs answered the phone on the second call, Client 1 said, "Gretchen he's free, come here."
29. Officer Jacobs asked Client 1 who was in the background telling Client 1 what to say, Client 1 responded, "No one." When Officer Jacobs asked Client 1 to put Respondent on the phone, Client 1 insisted Respondent was not there and continually repeated throughout the call that Respondent was not there.
30. In an interview with OPR Investigator Bowdish in September 2022, Respondent confirmed she sees Client 1 on Mondays and Fridays for billed mental health counseling.
31. Respondent also confirmed she sees Client 1 on Wednesdays and Saturdays to take Client 1 into the community, which is not a billed service.

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32. When interviewed by Investigator Bowdish, Respondent attempted to justify her contact with Client 1 beyond therapy sessions and their outings into the community together by stating that workers hired for Client 1 by her family member have not worked out, there is not currently taxi service for Client 1, and that Client 1 is a particularly difficult client about adhering to routines like the non-billed outings with Respondent.
33. Respondent provided a copy of Client 1's Treatment Plan to Investigator Bowdish.
34. The Treatment Plan identifies that Client 1 has bipolar disorder and provides, in full:

PRESENTING PROBLEM: Stabilize and maintain good mental health.

Goal 1: "I feel better when I get out in the community several times a week."

Objective: [Client] will get out in the community 3-4 times a week.

Intervention:

Gretchen Lewis will provide [Client] with support to visit her brother at his home in the community 1-2 times a week.

Gretchen Lewis will provide space for [Client] to discuss family-of-origin issues and/or other issues weighing on her mind. By [Client]'s self-report, she will experience less emotional distress.

Goal 2: "I am concerned about my health."

Objective: [Client] will follow treatment recommendations, maintain her medication regime, eat as healthy as possible and drink plenty of fluids.

Intervention:

Gretchen Lewis will provide [Client] with support to obtain medications and medically necessary items from the pharmacy 1-2 times a week.

Gretchen Lewis will assist [Client] in obtaining healthy dinners prepared for her by certain restaurants in the community.

Gretchen Lewis will monitor the effectiveness of [Client]'s medication based on her level of functioning.

Gretchen Lewis will use time spent with [Client] to help her develop reality-based cognitive messages that will increase her self-confidence in coping with fears and anxieties regarding her health.

[Client] will report positive gains regarding her health.

Boundary Issues with Clients re: Office Space

35. From October 2021 until mid-May 2022, Respondent practiced mental health counseling, alcohol abuse counseling and substance abuse counseling in a leased office in a building with other professional's offices.

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36. OPR received a complaint from another professional who also occupied the building regarding the use of Respondent's office and the building's common space by Respondent's clients.
37. Respondent's clients routinely accessed the building's common space and her office when Respondent was not there.
38. Respondent gave her client S.H. ("Client 2") permission to use her office address for Client 2's mail because Client 2 was homeless. Respondent told Client 2 to stop by her office a couple of times a week to retrieve his mail.
39. The building occupant witnessed Client 2 in Respondent's office sitting on the sofa and drinking beer while Respondent was not in the building.
40. Respondent gave Client 2 permission to sleep in her office on at least one occasion during the winter and Client 2 did so.
41. Respondent allowed a male and female who were in a relationship to sleep in the building for one night in the winter. Respondent stated the people were not her clients but she had helped them get housing.
42. On or about May 5, 2022, the building occupant witnessed another of Respondent's clients, a male with the last name starting with "C," in the common area of the building drinking alcohol, spitting, and carrying a long stick.
43. The building occupant has witnessed as many as five of Respondent's clients in the waiting room at the same time when Respondent was not in the building.
44. Respondent admitted to Investigator Bowdish that she does not lock her office.
45. When asked about confidential patient records, Respondent told the investigator she has been using handwritten paper notes but is moving to electronic notes, that she does not keep confidential records in her office, and that she carries confidential records with her in her briefcase every time she leaves the office.
46. During the day, Respondent keeps a clipboard with handwritten client session notes and then takes them home every night.

Boundary and Ethical Issues with Clients 3, 4, and 5

47. Respondent provided counseling services to an individual, M.M. ("Client 3").
48. Respondent conducted an assessment of Client 3 in 2012, then provided consistent counseling services to Client 3 between 2014 and 2020.
49. At various times when Client 3 brought other persons with her to counseling sessions, Respondent did not directly address confidentiality with Client 3 and instead would "try to take cues" from Client 3.

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50. Respondent did not obtain Client 3's informed consent to discuss Client 3 with others.
51. Between 2018 and 2020, Respondent communicated with Client 3 using Facebook Messenger on many occasions, messaging which showed the following:
 - a. Respondent provided rides to Client 3;
 - b. Respondent told Client 3 to go into her office when Respondent was not going to be there to pick up a signed form;
 - c. Respondent stated to Client 3, "you are a love;"
 - d. Respondent communicated with Client 3's significant other about Client 3 when Client 3 was not present;
 - e. Respondent gave Client 3 money for softball dues;
 - f. Respondent desired to surprise Client 3's roommate with a birthday gift and elicited Client 3's advice about what to get her roommate;
 - g. Respondent gave Client 3 unsolicited advice regarding Client 3's medications;
 - h. Respondent opined to Client 3 that smoking weed was "probably the best thing for [Client 3]" while Client 3 was waiting to get on ADHD medications.
52. In June 2020, Client 3 requested her records from Respondent. Respondent did not provide the records until December 2020.
53. Client 3 last saw Respondent for counseling in July 2020.
54. In December 2020, when Respondent provided Client 3 her records, Respondent inadvertently included a map of the family of another client, J.B. ("Client 4") who is mentioned below. The map includes names, relationships, and confidential information such as mental health issues and medical issues.
55. In 2019, Respondent also provided counseling services to J.B. ("Client 4"), who was Client 3's romantic partner when Respondent took Client 4 on as a client.
56. Respondent did not obtain informed consent to engage in counseling with both Clients 3 and 4 while they were in an intimate relationship.
57. In a Facebook Messenger message to Client 3, Respondent stated: "[Client 4] said she had the best New Years ever with you." This was something Client 4 said to Respondent while in counseling.
58. Between 2014 and 2019, Respondent also provided counseling services to a client, J.D. ("Client 5").
59. Client 5 and Client 3 were in a romantic relationship during a period of time when Respondent was the counselor for both of them.
60. Respondent did not obtain informed consent to engage in counseling with both Clients 5 and 3 while they were in an intimate relationship.

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61. On multiple occasions, Respondent went to Client 5's home for therapy sessions at night between 7:00 p.m. and 9:00 p.m.
62. Respondent did not do couples counseling with Clients 3 and 5 but told Investigator Bowdish when Respondent and Client 5 had sessions at Client 5's home and Client 3 was there, Client 3 might participate and provide her perspective.

Anger Management Assessment of Client 6 in 2022

63. In August 2022, Respondent was hired to conduct an assessment of a client, L.C. ("Client 6").
64. Client 6 was on probation for violating conditions of release arising from charges against him for domestic assault of his ex-wife.
65. As part of his probation, Client 6 was required to obtain an Anger Management Assessment.
66. Client 6's ex-wife had previously been Respondent's client in 2017. When his ex-wife was Respondent's client in 2017, Client 6 accompanied her to four of her sessions with Respondent.
67. In August 2022, Respondent met with Client 6 twice and wrote a one-paragraph "assessment" that included the following:

[Client 6] does not meet the criteria for Anger Management at this time. He, in fact, seems to be stuck in an abusive situation with his ex-wife. [Client 6] simply accepted the charges he was accused of because he was being kept from his children for so long with the courts as back-logged as they are. [Client 6] is again being kept from his children by his ex-wife for an unknown reason and he is handling this situation with no acts of aggression or anger. He is following all legal procedures to resolve this situation despite his internal heartbreak. [Client 6] will be voluntarily continuing to seek mental health counseling with me at this time due to his sadness and concern for his children who he has not seen for a month at this point despite a court order.

68. Respondent billed Medicaid for "Psychotherapy – 1 hour" for the two anger management assessment sessions in August 2022, and her notes for the sessions appear to be her standard handwritten SOAP notes.
69. Respondent continued to provide therapy for Client 6 after the assessment.
70. In response to OPR Investigator Bowdish's request for Respondent's therapy notes for Client 6, Respondent included notes from therapy sessions Client 6 attended with his ex-wife who was Respondent's client in 2017.

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Boundary and Ethical Issues with Clients 7 and 8

71. Respondent was the mental health counselor for Client 7 and Client 8.
72. As of the beginning of 2023, Client 7 had been a client of Respondent for more than one year.
73. As of early 2023, Client 8 had been a client of Respondent for between four and five years.
74. Clients 7 and 8 are domestic partners who live together in a rural area outside of Newport, Vermont. They do not have a car and rely upon Rural Community Transportation (RCT) for transportation.
75. On multiple occasions leading up to and during March 2023, Respondent picked up prescriptions for Clients 7 and 8 from the pharmacy.
76. On multiple occasions leading up to and during March 2023, Respondent did grocery shopping for Clients 7 and 8 at the food shelf.
77. On multiple occasions leading up to and during March 2023, Respondent took Clients 7 and 8 to Walmart to do their shopping.
78. Respondent assisted Clients 7 and 8 with obtaining housing.
79. During the relevant time, Clients 7 and 8 were experiencing problems with a neighbor and were concerned about their own safety.
80. Respondent encouraged Clients 7 and 8 to seek assistance from Umbrella, a non-profit domestic violence advocacy and resource organization located in Newport, Vermont.
81. On March 20, 2023, Respondent picked up Clients 7 and 8 at their home to take them to Umbrella for assistance with getting a restraining order against their neighbor.
82. On March 20, 2023, Client 7 had significant mobility issues due to a broken leg in a full cast. Client 7 used the assistance of either a wheelchair or a walker.
83. Client 7 did not bring her wheelchair on March 20th. She brought her walker.
84. Respondent informed Clients 7 and 8 that she would not be able to provide them with a ride home until later but did not specify a time.
85. Respondent dropped Clients 7 and 8 off in the parking lot of Umbrella at approximately 9:30 a.m.
86. It was snowing at the time, and Client 7 had significant difficulty getting up the wheelchair ramp with her walker. A staff member from Umbrella went outside to

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assist Client 7 and spent approximately 20 minutes helping Client 7 to get into the building.

87. The organization was not able to assist Clients 7 and 8 because their situation did not involve domestic violence.
88. Clients 7 and 8 had no means to leave Umbrella and texted Respondent at 10:45 a.m. inquiring when she would be able to provide them with a ride home, to which Respondent answered "tonight."
89. Clients 7 and 8 contacted another relative trying to find a ride, but without success.
90. Client 7 did not have her medications with her and missed her midday dose of Lorazepam, which she takes three times daily for anxiety.
91. When a staff member from Umbrella contacted Respondent about the situation, Respondent stated she felt she was being "thrown under the bus" and that Clients 7 and 8 knew she would not be able to provide an immediate ride to them. Respondent suggested that Clients 7 and 8 walk to her office and did not understand why Clients 7 and 8 had not done so.
92. Ultimately, Umbrella contacted RCT for transport. Typically, RCT requires 48 hours' notice, but agreed to provide a ride to Clients 7 and 8 home that day. Umbrella paid for the transport, which occurred at some point after 3:00 p.m.
93. On March 25, 2023, Respondent sent Client 7 a text about what transpired on March 20th, stating: "I'm afraid of you two throwing me under the bus again, you knew darn well I was not available to drive you home right away."

Medicaid Billing Issues for Client 7

94. Respondent billed Medicaid for four sessions with Client 7 after January 20, 2023, as follows:

a. Saturday, January 21, 2023	90834 Psychotherapy – 45-50 minutes
b. Monday, January 30, 2023	90837 Psychotherapy – 1 hour
c. Monday, February 6, 2023	90837 Psychotherapy – 1 hour
d. Monday, February 13, 2023	90837 Psychotherapy – 1 hour
95. On January 21, 2023, Respondent made a therapy note for Client 7 about "strategized the solution to client getting her meds."
96. Also on January 21, 2023, Respondent signed for Client 7's prescriptions at Walgreens in Newport.
97. On January 30, 2023, Respondent made a therapy note for Client 7 about Client's "continued pain and discomfort."

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98. Also on January 30, 2023, Respondent signed for Client 7's prescriptions at Kinney Drug in Newport.
99. On February 6, 2023, Respondent made a therapy note for Client 7 stating:

Helped client strategize around getting her medication picked-up at the pharmacy since she is completely helpless with her broken leg at this point. Helped client identify what a trigger it is for her to be without her medication for her panic attacks. In fact, she came close to having a panic attack before she was able to find a way to get her medication.
100. Also on February 6, 2023, Respondent signed for Client 7's prescriptions at Kinney Drug in Newport.
101. On February 13, 2023, Respondent made therapy notes which related to Client 7 being worried about surgery.
102. Also on February 13, 2023, Respondent signed for Client 7's prescriptions at Kinney Drug in Newport.

Non-Compliance with 2021 Order

103. The 2021 Order was a negotiated stipulation to resolve multiple complaints against Respondent.
104. Paragraph 62 of the Stipulation provided, "Respondent admits that the facts above are true and that the conditions below are necessary to protect the public."
105. The 2021 Order required Respondent to "within ninety (90) days following the entry of this Order, at her own expense, complete twelve (12) hours of pre-approved online courses, which must include education on ethics, boundaries, dual relationships, ethical considerations while practicing in a small community, and risk management."
106. Because the Order was entered on August 24, 2021, Respondent was required to complete the coursework by November 22, 2021.
107. On or around December 16, 2021, Respondent completed 6.25 hours of coursework.
108. Respondent submitted her certificate of completion for the remaining hours on or around December 8, 2022, over a year after the due date.

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109. The 2021 Order contained the following prohibition (p.8, Section B(2)):

Respondent shall be prohibited from working in any OPR licensed capacity (i.e. as a LCMH[C], an LADC or any combination of the two) for more than forty (40) hours per week during the conditioned period.

110. Per the 2021 Order, the conditioned period began on the date of entry, August 24, 2021, continued for a minimum of two years of full time LCMHC and/or LADC practice and remained in place until Respondent successfully petitioned for removal of conditions by demonstrating full compliance with the Order.

111. The 2021 Order also provided (p.10, Section B(12)):

Any violation of this Order may be a basis for additional disciplinary action. If a complaint of unprofessional conduct is made against Respondent during the term of this Order, this Order shall be automatically extended until the unprofessional conduct matter is concluded.

112. After the entry of the 2021 Order, Respondent routinely billed over 40 hours per week for providing counseling services to clients.

113. For example, in the month following the entry of the 2021 Order, Respondent billed over 50 hours for counseling sessions in the week of September 13, 2021 and again in the week of September 20, 2021.

114. In 2022, Respondent billed in excess of 40 hours per week for over 40 weeks.

115. In 2022, Respondent billed 50 hours or more for 26 weeks.

116. Respondent's highest billing week in 2022 was 60.5 hours.

117. From January 1, 2023 through September 30, 2023 billing, Respondent has billed for counseling in excess of 40 hours per week for over 30 weeks. Respondent's highest billing week during that time was 54.75 hours.

Violation One: 3 V.S.A. §129a(b)(1) Performance of unsafe or unacceptable patient or client care.

118. Paragraphs 2 through 117 are incorporated herein.

119. As a Licensed Clinical Mental Health Counselor in Vermont, Respondent is required to perform safe and acceptable patient or client care.

120. Paragraphs 3 through 5, 12 through 70 demonstrate Respondent performed unsafe and unacceptable patient or client care when she: continued in a dual relationship with poor or no boundaries with Client 1, provided a ride to Client 1 and left Client

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1 at a restaurant with no transportation and no ability to get home, attempted to coerce Patient 1 while Patient 1 was speaking to the police about Respondent, allowed patients' unstructured and unfettered access to the building where her practice is located so that numerous patients occupied the common area at the same time, failed to set appropriate boundaries with patients, engaged in dual relationships with Clients 3, 4, and 5, violated confidentiality with multiple patients, provided rides to Client 3, and engaged in messaging with Client 3 about matters other than scheduling appointments over Facebook messenger.

121. Paragraphs 71 through 102 demonstrate Respondent performed unsafe and unacceptable patient or client care when she: engaged in a dual relationship with Clients 7 and 8 with poor or no boundaries; provided a ride to Clients 7 and 8 and left them at an organization that was not able to provide assistance to them and while the Clients had no transportation, without scheduled medications, no ability to get home on a snowy day and while Client 7's mobility was severely limited by a broken leg; placed blame on Clients 7 and 8 in a text to Client 7 saying they "threw her under the bus" regarding the situation at Umbrella; and billed Medicaid for psychotherapy sessions with Client 7 that did not occur.
122. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because Respondent has engaged in a pattern of unprofessional conduct in violation of 3 V.S.A. §129a(b)(1).

Violation Two: 3 VSA 129a(b)(2) Failure to conform to essential standards of acceptable and prevailing practice.

123. Paragraphs 2 through 117 are incorporated herein.
124. Vermont law requires Licensed Clinical Mental Health Counselors to conform to the essential standards of acceptable and prevailing practice.
125. The Administrative Rules of the Board of Allied Mental Health Practitioners ("ARAMH") provide that the ethical codes of the National Board for Certified Counselors ("NBCC") and the American Counseling Association ("ACA") establish the standard of practice and fitness within the Allied Mental Health professions. *See* ARAMH Rule 6.6.
126. Essential standards of acceptable and prevailing practice for Licensed Clinical Mental Health Counselors in Vermont require that a counselor's "Primary Responsibility" is to "respect the dignity and promote the welfare of clients." *See* ACA Code of Ethics, Standard A.1.a.
127. Paragraphs 3 through 5, 12 through 70 demonstrate Respondent failed to respect the dignity and promote the welfare of clients when she: continued in a dual relationship with poor or no boundaries with Client 1, provided a ride to Client 1 and left Client 1 at a restaurant with no transportation and no ability to get home, attempted to coerce Patient 1 while Patient 1 was speaking to the police about Respondent, allowed patients unstructured and unfettered access to the building

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where her practice is located so that numerous patients occupied the common area at the same time, failed to set appropriate boundaries with patients, engaged in a dual relationships with Clients 3, 4, and 5, violated confidentiality with and of multiple patients, provided rides to Client 3, and provided an anger management "assessment" to Client 6, who perpetrated domestic assault against his ex-wife, who had previously been Respondent's client.

128. Paragraphs 71 through 102 demonstrate Respondent failed to respect the dignity and promote the welfare of clients when she: engaged in a dual relationship with Clients 7 and 8 with poor or no boundaries; provided a ride to Clients 7 and 8 and left them at an organization that was not able to provide assistance to them and while the Clients had no transportation, without scheduled medications, no ability to get home on a snowy day and while Client 7's mobility was severely limited by a broken leg; placed blame on Clients 7 and 8 in a text to Client 7 saying they "threw her under the bus" regarding the situation at Umbrella; and billed Medicaid for psychotherapy sessions with Client 7 that did not occur.
129. Essential standards of acceptable and prevailing practice for Licensed Clinical Mental Health Counselors in Vermont require counselors to "avoid harming their clients" and "to minimize or to remedy unavoidable or unanticipated harm." See ACA Code of Ethics, Standard A.4.a.
130. Paragraphs 12 through 34 and 47 through 62 demonstrate Respondent failed to avoid harming Clients 1, 3, 4 and 5 by failing to maintain professional boundaries.
131. Paragraphs 71 through 102 demonstrate Respondent failed to avoid harming Clients 7 and 8 by failing to maintain professional boundaries.
132. Essential standards of acceptable and prevailing practice for Licensed Clinical Mental Health Counselors in Vermont require that "counselors consider the risks and benefits of extending current counseling relationships beyond conventional parameters....In extending these boundaries, counselors take appropriate professional precautions such as informed consent, consultation, supervision, and documentation to ensure that judgment is not impaired and no harm occurs." See ACA Code of Ethics, Standard A.6.b. Such boundary extensions must be documented and, when unintentional harm occurs, the counselor must attempt to remedy such harm. See ACA Code of Ethics, Standard A.6.c.
133. Paragraphs 3 through 5, 12 through 70 demonstrate that Respondent failed to consider the risks and benefits of extending counseling relationships beyond conventional parameters with Clients 1, 2, 3, 4, and 5, failed to take appropriate professional precautions to ensure that her judgment was not impaired and no harm occurred, and failed to document those extensions, including the rationale, the potential benefits and the anticipated consequences for clients and others.
134. Paragraphs 71 through 102 demonstrate that Respondent failed to consider the risks and benefits of extending counseling relationships beyond conventional parameters with Clients 7 and 8, failed to take appropriate professional precautions to ensure

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that her judgment was not impaired and no harm occurred, failed to document those extensions, including the rationale, the potential benefits and the anticipated consequences for clients and others, and failed to accept responsibility for her own role in leaving the clients at an organization that could not provide them assistance, with no way of walking elsewhere because Client 7 had significant mobility issues and it was snowing, no access to scheduled medication, and no means of transportation home.

135. Essential standards of acceptable and prevailing practice for Licensed Clinical Mental Health Counselors in Vermont require that when providing "counseling services to two or more persons who have a relationship, the counselor clarifies at the outset which person or persons are clients and the nature of the relationships the counselor will have with each involved person. If it becomes apparent that the counselor may be called upon to perform potentially conflicting roles, the counselor will clarify, adjust, or withdraw from roles appropriately." *See ACA Code of Ethics, Standard A.8.*
136. Paragraphs 47 through 62 demonstrate that Respondent failed to directly address or clarify with Clients 3, 4, and 5 which persons were clients, the nature of the relationships the counselor would have with involved persons, and how Respondent would manage counseling romantic partners when conflicts in their relationships arose. Respondent further failed to obtain appropriate informed consent from the clients about sharing of information and one client attending a session between Respondent and another client.
137. Paragraphs 63 through 70 demonstrate that Respondent failed to get informed consent from Client 6's ex-wife, who had been a client of Respondent and who was the victim of domestic assault perpetrated by Client 6.
138. Essential standards of acceptable and prevailing practice for Licensed Clinical Mental Health Counselors in Vermont require counselors to protect confidentiality, which includes a client's identity. Counselors disclose information only with appropriate consent or with sound legal or ethical justification. *See ACA Code of Ethics, Standard B.1.c.*
139. Paragraphs 35 through 62 demonstrate that Respondent failed to conform to the essential standards of acceptable and prevailing practice by failing to protect confidentiality when Respondent: allowed patients unstructured access to her building and office so that numerous patients occupied the common area at the same time; revealed confidential client information to other clients without consent; failed to obtain informed consent from clients when other persons attended confidential client sessions.
140. NCCs shall work collaboratively with clients in the creation of written plans of treatment that offer attainable goals and use appropriate techniques consistent with client's psychological and physical needs and abilities. *See National Board of Certified Counselors ("NBCC") Code of Ethics, Directive 73.*

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141. Paragraphs 12 through 34 demonstrate Respondent has failed to work with Client 1 to create a legitimate mental health treatment plan attainable through Respondent's provision of psychotherapy services to Client 1.
142. NCCs shall discuss service termination with clients when there is a reasonable belief that the clients are no longer benefiting from or are unlikely to benefit from future services. *See* NBCC Code of Ethics, Directive 76.
143. Paragraphs 3 through 34 demonstrate that Respondent has failed to discuss termination of services with Client 1 when circumstances demonstrate Client 1 is being harmed by Respondent's continued provision of mental health services to Client 1.
144. Essential standards of acceptable and prevailing practice require a Licensed Clinical Mental Health Counselor to "seek professional assistance or withdraw from the practice of counseling if their mental or physical condition makes it unlikely that the counselor will be able to provide appropriate services." NBCC Code of Ethics, Directive 23.
145. Essential standards of acceptable and prevailing practice for Licensed Clinical Mental Health Counselors (ACA Code of Ethics, Standard C.2.g) require:

Counselors monitor themselves for signs of impairment from their own physical, mental, or emotional problems and refrain from offering or providing professional services when impaired. They seek assistance for problems that reach the level of professional impairment, and, if necessary, they limit, suspend, or terminate their professional responsibilities until it is determined that they may safely resume their work.

146. Paragraphs 3 through 117 above demonstrate that Respondent has failed monitor herself for signs of impairment and to withdraw from the practice of counseling despite significant and repeated ethical and boundary violations with multiple clients over the course of time that demonstrate Respondent is not able to provide appropriate psychotherapy services to clients that conform to the essential standards of acceptable and prevailing practice.
147. The acts, omissions, and/or circumstances described above constitute grounds for discipline because Respondent has engaged in a pattern of unprofessional conduct in violation of 3 V.S.A. §129a(b)(2).

Violation Three: 26 V.S.A. §3271(a)(6) Entering into an additional relationship with a client, supervisee, research participant, or student that might impair the licensed clinical mental health counselor's objectivity or otherwise interfere with the clinical mental health counselor's professional obligations.

148. Paragraphs 2 through 117 above are incorporated herein.

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149. Vermont law prohibits a Licensed Clinical Mental Health Counselor from entering an additional relationship with a client that might impair the counselor's objectivity or interfere with the counselor's professional obligations.
150. Paragraphs 3 through 33 and 47 through 62 demonstrate that Respondent entered additional relationships with Clients 1, 3, 4 and 5 which impaired Respondent's objectivity and otherwise interfered with Respondent's professional obligations.
151. Paragraphs 71 through 102 demonstrate that Respondent entered additional relationships with Clients 7 and 8 which impaired Respondent's objectivity and otherwise interfered with Respondent's professional obligations.
152. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because Respondent has engaged in a pattern of unprofessional conduct in violation of 26 V.S.A. §3271(a)(6).

Violation Four: 3 V.S.A. §129a(a)(3) Failure to comply with provisions of federal or State statutes or rules governing the practice of the profession, incorporating 45 C.F.R. §164.502.

153. Paragraphs 2 through 117 above are incorporated herein.
154. The Health Insurance Portability and Accountability Act of 1996 ("HIPAA") is a federal law which prohibits a health care provider from accessing or using a patient's protected health information for a purpose other than treatment, payment, or health care operations of the Facility, except when authorized by the individual or as required by law. *See* 45 C.F.R. §164.502.
155. Respondent is subject to HIPAA as a health care provider in her capacity as a mental health counselor. *See* 45 C.F.R. § 160.103.
156. Paragraphs 47 through 62 demonstrate that Respondent repeatedly violated federal law by revealing confidential client information to others without proper authorization or permitted purpose.
157. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because the Respondent has engaged in a pattern of unprofessional conduct in violation of 3 V.S.A. §129a(a)(3).

Violation Five: 3 V.S.A. §129a(a)(8) Failure to make available promptly to a person using professional health care services, that person's representative, or succeeding health care professionals or institutions, upon written request and direction of the person using professional health care services, copies of that person's records in the possession or under the control of the licensed practitioner.

158. Paragraphs 2 through 117 above are incorporated herein.

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- 159. Vermont law requires a Licensed Clinical Mental Health Counselor to promptly make client records available to clients upon written request.
- 160. Paragraphs 52 through 54 demonstrate Respondent failed to promptly provide Client 3 her records upon written request.
- 161. The act(s), omission(s), and/or circumstance(s) described above constitute grounds for discipline because the Respondent has engaged in a pattern of unprofessional conduct in violation of 3 V.S.A. §129a(a)(8).

Violation Six: 3 V.S.A. §129a(a)(26) Failing to maintain professional boundaries; or violating a patient, client, or consumer's reasonable expectation of privacy.

- 162. Paragraphs 2 through 117 above are incorporated herein.
- 163. In Vermont, a Licensed Clinical Mental Health Counselor must maintain professional boundaries and must observe and respect a patient, client, or consumer's reasonable expectation of privacy.
- 164. Paragraphs 2 through 62 above demonstrate that Respondent failed to maintain professional boundaries with her clients.
- 165. Paragraphs 71 through 102 demonstrate that Respondent failed to maintain professional boundaries with Clients 7 and 8.
- 166. Paragraphs 2 through 62 above demonstrate that Respondent violated clients' reasonable expectations of privacy on many occasions.
- 167. The acts, omissions, and/or circumstances described above constitute grounds for discipline because Respondent has engaged in a pattern of unprofessional conduct in violation of 3 V.S.A. §129a(a)(26).

Violation Seven: 3 V.S.A. §129a(a)(15) Failing to exercise independent professional judgment in the performance of licensed activities when that judgment is necessary to avoid action repugnant to the obligations of the profession.

- 168. Paragraphs 2 through 117 above are incorporated herein.
- 169. In Vermont, a Licensed Clinical Mental Health Counselor must exercise independent professional judgment in the performance of licensed activities when that judgment is necessary to avoid action repugnant to the obligations of the profession.
- 170. Paragraphs 2 through 62 above demonstrate that Respondent failed to exercise independent professional judgment when she: left Client 1 at a local restaurant when Client 1 had no transportation to return home, no phone, no coat, no taxi service; attempted to coerce Client 1 while Client 1 was speaking to police about Respondent; allowed clients to congregate in her shared professional building;

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allowed Client 2 and others to sleep in the building where her office was located; allowed Client 2 to use Respondent's office as his address for mail; disclosed confidential client information without consent; engaged in individual counseling with multiple romantic partners without informed consent; engaged in dual relationships with Clients 1, 3, 4 and 5; failed to obtain informed consent from clients and consider boundary extension ramifications before proceeding with therapy in the presence of others; conducted therapy in clients' homes during nighttime hours; and engaged in messaging with Client 3 about matters other than scheduling appointments over Facebook messenger; and provided an anger management "assessment" for Client 6, who had perpetrated domestic assault against his ex-wife, Respondent's former client.

171. Paragraphs 71 through 102 demonstrate Respondent failed to exercise independent professional judgment in the performance of licensed activities when that judgment was necessary to avoid actions repugnant to the obligations of the profession when she: engaged in a dual relationship with Clients 7 and 8 with poor or no boundaries; provided a ride to Clients 7 and 8 and left them at an organization that was not able to provide assistance to them and while the Clients had no transportation, without scheduled medications, no ability to get home on a snowy day and while Client 7's mobility was severely limited by a broken leg; placed blame on Clients 7 and 8 in a text to Client 7 saying they "threw her under the bus" regarding the situation at Umbrella; and billed Medicaid for psychotherapy sessions with Client 7 that did not occur.
172. Paragraphs 103 through 117 above demonstrate that Respondent failed to exercise independent judgment in the performance of licensed activities when that judgment was necessary to avoid actions repugnant to the obligations of the profession when Respondent blatantly and routinely violated restrictions imposed on her license by the Board and with her agreement and when she agreed the conditions were necessary to protect the public because she engaged in unprofessional conduct on multiple occasions prior to the entry of the 2021 Order.
173. The acts, omissions, and/or circumstances described above constitute grounds for discipline because Respondent has engaged in a pattern of unprofessional conduct in violation of 3 V.S.A. §129a(a)(15).

Violation Eight: 3 V.S.A. §129a(a)(4) Failing to comply with an order of the board or violating any term or condition of a license restricted by the Board.

174. Paragraphs 2 through 119 above are incorporated herein.
175. In Vermont, a Licensed Clinical Mental Health Counselor must comply with an order of the board and any term or condition of a license restricted by the Board.
176. Paragraphs 103 through 117 above demonstrate that Respondent routinely violated the 2021 Order by working more than 40 hours per week and by failing to complete coursework within the specified time.

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177. The acts, omissions, and/or circumstances described above constitute grounds for discipline because Respondent has engaged in a pattern of unprofessional conduct in violation of 3 V.S.A §129a(a)(4).

Understandings

178. Respondent understands that the Board must review and accept the terms of the Consent Order. If the Board rejects any portion, the entire Stipulation and Consent Order shall be null and void.
179. Respondent specifically waives any claims that any disclosures made to the Board during its review of this agreement have prejudiced Respondent's rights to a fair and impartial hearing if this agreement is not accepted by the Board and proceeds to a contested hearing.
180. Respondent has read and reviewed this entire document and agrees that it contains the entire agreement between the parties.
181. Respondent is not under the influence of any drugs or alcohol at the time Respondent signs this Stipulation and Consent Order.
182. Respondent voluntarily enters into this agreement after the opportunity to consult with legal counsel and is not being coerced by anyone into signing this Stipulation and Consent Order.
183. Respondent voluntarily waives Respondent's right to a contested hearing before the Board and waives any right to appeal from this Stipulation and Consent Order.
184. Respondent agrees that the Order set forth below may be entered by the Board.

ORDER

Based on the Stipulation above, it is **ORDERED AND ADJUDGED** as follows:

- A. The Board of Allied Mental Health Practitioners hereby **INDEFINITELY SUSPENDS** Respondent's license to practice as a Licensed Clinical Mental Health Counselor for a minimum period of two (2) years beginning December 31, 2023. Respondent must petition for reinstatement but may not do so for a period of two years. No sooner than forty-five (45) days prior to requesting reinstatement, Respondent must, at her own cost:

1. Obtain an independent mental health evaluation by a pre-approved doctorate-level licensed professional with more than ten years' experience ("Independent Evaluator") who has verified receipt of this Stipulation and Consent Order, the 2021 Order, and the Supervisor Reports of Robert Munger. The independent evaluator must verify that he/she has no personal or professional background or history with Respondent. The evaluation report shall meet the independent evaluation requirements posted on OPR's website. The results of the

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Independent Evaluation must indicate that Respondent is fit to practice and does not present a risk to the safety, health, or welfare of Respondent's potential patients, as well as any recommendations that would ensure Respondent is safe to practice. The evaluation must be submitted to the Enforcement Unit Case Manager.

2. If recommended by an Independent Evaluator, enter into a treating professional agreement and mental health treatment plan with a pre-approved treating professional and follow all recommendations.
 3. Successfully complete (i.e., receive a passing score, if applicable) fifteen (15) hours of pre-approved coursework (which may also count toward the coursework required for the Stipulation and Consent Order regarding Respondent's LADAC license) on Ethics, Boundaries, and relationships with clients. The coursework must be pre-approved by the Enforcement Case Manager. Respondent shall submit the certificates of completion, completed assignment book(s) and quiz results, if applicable, to the Enforcement Case Manager. The coursework may not be counted toward any continuing education requirement of licensure.
 4. Respondent's petition for reinstatement must include a detailed personal statement written by Respondent that reflects on the conduct that has given rise to the discipline, what she has learned through the disciplinary process and the coursework she has completed, as well as a full accounting of why she believes she engaged in the conduct described above, what steps she has taken to rehabilitate, why adhering to ethical practice is essential in counseling practice, what steps and plan she believes will ensure that she will not engage in such conduct in the future. Respondent shall also submit documentation and letters of support with the petition.
- B. If reinstated, Respondent's license **SHALL BE CONDITIONED** pursuant to whatever terms the Board finds reasonable at the time of Respondent's reinstatement request, taking into account the independent evaluation, the facts above, the personal statement and anything else the Board deems relevant at that time. Conditions may include treatment/counseling, additional/ongoing coursework, supervision, practice setting requirements limiting Respondent to practice only at a facility where supervision is available, limitations as to the number of hours per week Respondent may practice, and employer reports.
- C. Notwithstanding any provision above, the Respondent must continue to meet all Board requirements for maintaining a license, license renewal and license reinstatement.
- D. This Stipulated and Consent Order is a matter of public record and may be reported to other licensing authorities as provided in 3 V.S.A. §129(a).

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- E. This Stipulation and Consent Order will remain part of Respondent's licensing file and may be used for purposes of determining sanctions in any future disciplinary matter.
- F. This Stipulation and Consent Order shall be applicable as a matter of law to all other licenses issued by the Board of Allied Mental Health Practitioners, as provided in 3 V.S.A. §129(l).

AGREED TO:

STATE OF VERMONT
SECRETARY OF STATE

Dated: 11/15/23

By: /s/ Jennifer B. Colin
Jennifer B. Colin
State Prosecuting Attorney

RESPONDENT

Dated: _____

By: _____
Gretchen E. Lewis

APPROVED AS TO FORM:

COUNSEL FOR RESPONDENT

By: _____
Jennifer McDonald
Attorney for Gretchen Lewis

APPROVED AND SO ORDERED:

VERMONT BOARD OF ALLIED MENTAL
HEALTH PRACTITIONERS

Dated: 11-16-23

By:

DocuSigned by:
Mary D. LCMAC
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Board Chairperson

Date of Entry: 11-17-23

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- E. This Stipulation and Consent Order will remain part of Respondent's licensing file and may be used for purposes of determining sanctions in any future disciplinary matter.
- F. This Stipulation and Consent Order shall be applicable as a matter of law to all other licenses issued by the Board of Allied Mental Health Practitioners, as provided in 3 V.S.A. §129(l).

AGREED TO:

STATE OF VERMONT
SECRETARY OF STATE

Dated: _____

By: _____
Jennifer B. Colin
State Prosecuting Attorney

Dated: 11/16/2023

By: _____
RESPONDENT
Gretchen E. Lewis

APPROVED AS TO FORM:

COUNSEL FOR RESPONDENT

By: _____
Jennifer McDonald
Attorney for Gretchen Lewis

APPROVED AND SO ORDERED:

VERMONT BOARD OF ALLIED MENTAL
HEALTH PRACTITIONERS

Dated: _____

By: _____
Board Chairperson

Date of Entry: _____

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